

INZOLAM TABLET 0.25 MG

DESCRIPTION:

An oval, scored tablet blue in colour with markings 'DUO 861' and 'd/d'.

COMPOSITION:

Each tablet contains Triazolam 0.25mg.

PHARMACODYNAMICS:

Triazolam is a potent short-acting benzodiazepines with general properties similar to those of diazepam.

PHARMACOKINETICS:

Triazolam is rapidly and nearly completely absorbed from the gastrointestinal tract, peak plasma concentrations being achieved within 2 hours of administration by mouth. Triazolam has a plasma elimination half-life ranging from 1.5 to 5.5 hours. It is reported to be about 89% bound to plasma proteins. Hydroxylation of Triazolam in the liver is mediated by the cytochrome P450 isozyme CYP3A4. Triazolam is excreted in the urine mainly in the form of its conjugated metabolites with only small amounts appearing unchanged.

INDICATION:

Inzolam is useful in the management of patients with transient and short term insomnia. It is also useful as a short term adjunctive treatment in the management of selected patients with long-term insomnia.

RECOMMENDED DOSAGE:

It is important to individualized dosage of Triazolam in patients within various population groups in order to obtain maximum therapeutic effect while using the smallest effective dose. The recommended dose for most adults is 0.25 mg before retiring. A dose of 0.125 mg may be found sufficient for some patients (e.g. low body weight). A dose of 0.5 mg should be used only for exceptional patients who do not respond adequately to a trial of a lower dose since the risk of several adverse reactions increases with the size of the dose administered.

A dose of 0.5mg should not be exceeded. In geriatric and/or debilitated patients the recommended dosage range is 0.125 mg to 0.25 mg.

Therapy should be initiated at 0.125 mg in this group and the 0.25 mg dose should be used only for exceptional patients who do not respond to a trial of the lower dose. A total dose of 0.25 mg should not be exceeded in these patients.

ROUTE OF ADMINISTRATION:

Oral

CONTRAINDICATIONS:

Triazolam is contraindicated with ketoconazole, itraconazole, and nefazodone, medications that significantly impair the oxidative metabolism mediated by cytochrome P450 3A (CYP 3A), pregnancy, lactation (breast feeding), depressed patients with suicidal tendencies, hypersensitivity to benzodiazepines, Myasthenia gravis, chronic obstructive airways disease with incipient respiratory failure.

WARNINGS & PRECAUTIONS:

Warning: May be habit forming on prolonged used

Anaphylaxis (Severe allergic reaction) and angioedema (severe facial swelling), which can occur as early as the first time the product is taken.

Complex Sleep – Related behaviours which may include sleep driving, making phone calls, preparing and eating food (while asleep).

Caution to patients not to take Inzolam in circumstances where a full night's sleep and clearance of the drug from the body are not possible before they would again need to be active and functional, e.g. an overnight flight of less than 7 – 8 hours, since amnesic episodes have been reported in such situations.

Depression, Psychosis and Schizophrenia: Triazolam is not recommended as primary therapy for patients with depression and psychosis. In such conditions, psychiatric assessment and supervision are necessary if benzodiazepines are indicated. Benzodiazepine may increase depression in some patients and may contribute to deterioration in severely disturbed schizophrenics with confusion and with withdrawal. Suicidal tendencies may be present or uncovered and protective measures may be required.

Paradoxical Reactions: Paradoxical reactions such as acute rage, stimulation or excitement may occur, should such reactions occur, Triazolam should be discontinued.

Geriatric or Debilitated Patients: Such patients may be particularly susceptible to the sedative effects of benzodiazepines and associated giddiness, ataxia and confusion, which may increase the possibility of a fall. For this reason, the dosage should be limited to the smallest effective amount to preclude such effects. The systemic availability of oral Triazolam is increased in geriatric patients probably due to a diminished first-pass hepatic metabolism.

Impaired Renal/Liver Function: Patients with impaired renal or liver function should use benzodiazepine medication with caution and a reduction in dosage, or decision not to prescribe, may be necessary in such patients. In rare instances some patients taking benzodiazepines have developed blood dyscrasias, and some have had elevations of liver enzymes. As with other benzodiazepines, periodic blood counts and liver function tests are recommended. Caution must be used in treating patients with impaired hepatic function, severe pulmonary insufficiency, or sleep apnoea.

Impaired Respiratory Function: Caution in the use of Triazolam is recommended in patients with respiratory depression. In patients with chronic obstructive pulmonary disease, benzodiazepines can cause increased arterial carbon dioxide tension and decreased oxygen tension. In patients with compromised respiratory function, respiratory depression and apnoea have been reported infrequently.

Hypotension: Although hypotension may occur only rarely, benzodiazepines should be administered with caution to patients in whom a drop in blood pressure might lead to cardiac or cerebral complications. This is particularly important in the elderly patient.

Abuse: Caution must be exercised in administering Triazolam to individuals known to be addiction prone, or those whose history suggests they may increase the dosage on their own initiative. It is desirable to limit repeat prescriptions without adequate medical supervision.

Dependence: The use of benzodiazepines may lead to dependence as defined by the presence of a withdrawal syndrome on discontinuation of the drug. Tolerance as defined by a need to increase the dose in order to achieve the same therapeutic effect seldom occurs in patients receiving the recommended dose under medical supervision. Tolerance to sedation may occur with benzodiazepines, especially in those with drug seeking behaviour. Tolerance may develop during 1 to 2 weeks of therapy. Withdrawal symptoms, similar in character to those noted with barbiturates and alcohol have occurred following abrupt discontinuance with benzodiazepines, including Triazolam. These symptoms range from insomnia, anxiety, dysphoria, palpitations, panic attacks, vertigo, myoclonus, akinesia, hypersensitive to light, sound and touch, abnormal body sensations (eg. feelings of motion, metallic taste), depersonalization, derealisation, delusional beliefs, hyperreflexia and loss of short term memory, to a major syndrome which may include convulsions, seizures, tremor, abdominal and muscle cramps, confusional state, delirium, hallucinations, hyperthermia, psychosis, vomiting and sweating. Such manifestations of withdrawal, especially the more serious ones are more common in patients who have received excessive doses over a prolonged period. However, withdrawal symptoms have been reported following abrupt discontinuation of benzodiazepines taken continuously at therapeutic levels. Accordingly, Triazolam should be terminated by tapering the dose to minimize occurrence of withdrawal symptoms. Patients should be advised to consult with their physician before either increasing the dose or abruptly discontinuing the medication. Rebound phenomena have been described in the context of benzodiazepine use. Rebound insomnia and anxiety mean an increase in the severity of these symptoms beyond pre-treatment levels following the cessation of benzodiazepines. Rebound phenomena in general possible reflect three-emergence of pre-existing symptoms combined with withdrawal symptoms described earlier. Some patients prescribed benzodiazepines with very short half-lives (in the order of 2 to 4 hours) may experience relatively mild rebound symptoms between their regular doses. Withdrawal/rebound symptoms may follow the use of high doses for relatively short periods.

Acute Narrow-Angle Glaucoma: Caution should be used in the treatment of patients with acute-narrow glaucoma (because of atropine-like side effects).

Epilepsy: Abrupt withdrawal of benzodiazepines in persons with convulsive disorders may be associated with a temporary increase in the frequency and/or severity of seizures. Patients with convulsive disorder should not be abruptly withdrawn from Triazolam.

Use in Paediatrics: Benzodiazepines and other hypnotic drugs may impair mental alertness in children. Triazolam is not recommended as safety and effectiveness in patients under the age of 18 has not been established.

Risks from Concomitant Use with Opioids

Profound sedation, respiratory depression, coma, and death may result from the concomitant use of Inzolam with opioids. Observational studies have demonstrated that concomitant use of opioids and benzodiazepines increases the risk of drug-related mortality compared to use of opioids alone. Because of these risks, reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate.

If the decision is made to newly prescribe a benzodiazepine and an opioid together, prescribe the lowest effective dosages and minimum durations of concomitant use.

If the decision is made to prescribe a benzodiazepine in a patient already receiving an opioid, prescribe a lower initial dose of the benzodiazepine than indicated in the absence of an opioid, and titrate based on clinical response.

If the decision is made to prescribe an opioid in a patient already taking a benzodiazepine, prescribe a lower initial dose of the opioid, and titrate based on clinical response.

Follow patients closely for signs and symptoms of respiratory depression and sedation. Advise both patients and caregivers about the risks of respiratory depression and sedation when Inzolam is used with opioids. Advise patients not to drive or operate heavy machinery until the effects on concomitant use of the opioid have been determined. Screen patients for risk of substance use disorders, including opioid abuse and misuse, and warn them of the risk for overdose and death associated with the use of opioids (*see Drug Interactions*).

INTERACTION WITH OTHER MEDICAMENTS:

The benzodiazepines, including Triazolam, produce additive CNS depressant effects when co-administered with other medication which themselves produce CNS depression e.g. barbiturates, alcohol, sedatives, tricyclic antidepressants, non-selective MAO inhibitors, phenothiazines and other antipsychotics, skeletal muscle relaxants, antihistamines, narcotic analgesics and anaesthetics. Triazolam undergoes oxidative metabolism, and consequently may interact with disulfiram or cimetidine resulting in increased plasma levels of Triazolam. Increased levels of Triazolam have also been observed with co-administration of erythromycin. Patients should be observed closely for evidence of enhanced benzodiazepine response during concomitant treatment with disulfiram, verapamil, diltiazem cimetidine, erythromycin or other macrolide antibiotics such as clarithromycin; some patients may require a reduction in the dosage of Triazolam. In the cases of nefazodone, ketoconazole, and itraconazole, the

magnitude of the interaction is such that coadministration of triazolam with these drugs is not recommended. In general, caution should be used when triazolam is used in combination with a drug that is known to inhibit cytochrome P450IIA4. The pharmacokinetics of triazolam may be altered by co-administration of grapefruit juice. The anticholinergic effects of other drugs, including atropine and similar drugs, antihistamines and antidepressants may be potentiated when taken in conjunction with benzodiazepines. Interactions have been reported between some benzodiazepines and anticonvulsants, with changes in the serum concentration of the benzodiazepine or the anticonvulsant. It is recommended that patients be observed for altered responses when benzodiazepines and anticonvulsants are prescribed together, and that serum level monitoring of the anticonvulsant be performed more frequently. Minor EEG changes, usually low voltage fast activity, of no known clinical significance, has been reported with benzodiazepine administration.

Opioids

Due to additive pharmacologic effect, the concomitant use of opioids with benzodiazepines increases the risk of respiratory depression, profound sedation, coma and death.

The concomitant use of opioids and benzodiazepines increases the risk of respiratory depression because of actions at different receptor sites in the central nervous system that control respiration. Opioids interact primarily at μ -receptors, and benzodiazepines interact at GABA_A sites. When opioids and benzodiazepines are combined, the potential for benzodiazepines to significantly worsen opioid-related respiratory depression exists.

Reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate. (*see Warnings and Precautions*).

Limit dosage and duration of concomitant use of benzodiazepines and opioids, and follow patients closely for respiratory depression and sedation.

USE IN PREGNANCY & LACTATION:

Use in Pregnancy: Triazolam is contra-indicated in pregnant women. Benzodiazepines cross the placenta and may cause hypotonia, respiratory depression and hypothermia in the newborn infant. Continuous treatment during pregnancy and administration of high doses in connection with delivery should be avoided. Withdrawal symptoms in newborn infants have been reported with this class of drug.

Use in Lactation: Triazolam is contra-indicated in breast feeding mothers. Human studies on the excretion of Triazolam in breast milk have not been performed. Studies in rats have, however, indicated that Triazolam and its metabolites are excreted in breast milk. Benzodiazepines generally show increased toxicity in neonates, and the excretion of benzodiazepines in breast milk may cause drowsiness and/or feeding difficulties in the infant.

SIDE EFFECTS:

The most common adverse effects are drowsiness/sedation, dizziness/lightheadedness, headache and impaired coordination/impaired equilibrium/ataxia, nervousness/anxiety, nausea/vomiting, tiredness/lethargy, concentration difficulty. The incidence and severity of these events is generally dose-related. Severe sedation and impaired coordination are indicative of drug intolerance and overdose. Adverse reactions which have been reported less frequently are depression, euphoria, confusional states, memory impairment, anterograde amnesia, visual disturbances, pains/cramps, palpitations/tachycardia, hiccups, taste alterations, skin rash, pruritis, paraesthesia, constipation/diarrhea. Paradoxical reactions such as acute rage, stimulation, excitement, agitation, sleep disturbances, hallucinations, disorientation in time or place, paranoia and other adverse behavioural or psychotic effects have been reported with benzodiazepines, including Triazolam. Additionally, somnambulism, hostility, aggressiveness, bizarre behaviour, depersonalization and abnormal dreams have been reported.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Because of the potency of triazolam, some manifestations of overdose may occur at 2 mg, four times the maximum recommended therapeutic dose (0.5 mg). Overdosage of benzodiazepines is usually manifested by degrees of central nervous system depression, ranging from drowsiness to coma. In mild cases symptoms include drowsiness, mental confusion and lethargy. In more serious cases symptoms include ataxia, hypotonia, hypotension, respiratory depression, coma and very rarely death.

Treatment: In the management of overdose with any medication it should be borne in mind that multiple agents may have been taken. Following overdose with oral benzodiazepines, vomiting should be induced (within one hour) if the patient is conscious, or gastric lavage undertaken with the airways protected if the patient is comatose. If there is no advantage in emptying the stomach, activated charcoal should be given to reduce absorption. Hypotension and respiratory depression should be managed according to general principles. Haemoperfusion and haemodialysis are not useful in benzodiazepine intoxication. The benzodiazepine antagonist flumazenil may be useful in hospitalized patients for the reversal of acute benzodiazepine effects. Please consult the flumazenil product information prior to use. Experiments in animals have indicated that cardiopulmonary collapse can occur with massive intravenous doses of triazolam (over 100 mg/kg, more than 10,000 times the maximum daily human dose). This could be reversed with positive mechanical respiration and the intravenous infusion of noradrenaline acid tartrate or metaraminol. The oral LD₅₀ in mice is greater than 1000 mg/kg and in rats is greater than 5000 mg/kg.

STORAGE CONDITIONS:

Store below 30°C. Protect from light. Keep out of reach of children. JAUHKAN DARIPADA KANAK-KANAK.

SHELF LIFE:

Please refer to outer package.

PACK SIZE:

Pack in blister pack of 10 x 10's and 50 x 10's tablets in a box.

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD
Lot 2599, Jalan Seruling 59 Kawasan 3,
Taman Klang Jaya, 41200 Klang, Selangor, MALAYSIA.

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